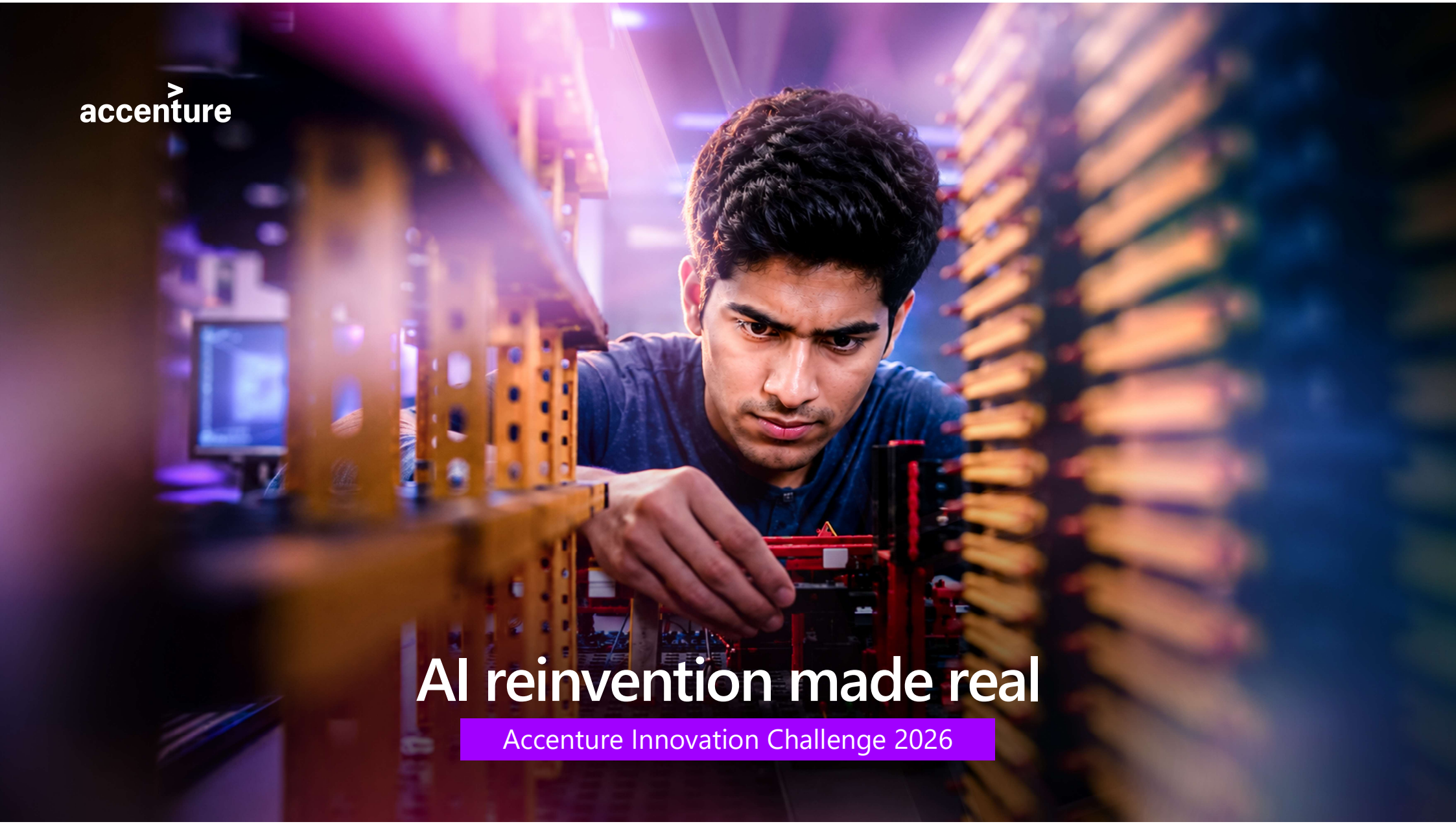

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AI reinvention made real
Accenture Innovation Challenge 2026

Instructions

Reference slide– Remove before submission

1. Use the given template for your idea submission.

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Team details

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All fields are mandatory

The Opportunity Behind PatientTriage.ai

PatientTriage.ai

THE GAP IN TODAY'S SYSTEMS

- EHRs primarily store and present information
- Traditional triage largely creates an initial priority
- Hospital dashboards provide visibility, not orchestration
- Operational decisions are often distributed across teams

WHAT HOSPITALS ACTUALLY NEED

- Continuous visibility of patient risk
- Dynamic prioritization as conditions change
- Better coordination of beds, doctors and treatment spaces
- Faster identification of operational bottlenecks
- Human decision-making supported by relevant information

OUR BUSINESS OPPORTUNITY

- Build an intelligence layer for Emergency Departments
- Connect clinical risk with operational resources
- Start with triage and expand into patient-flow management
- Deliver value using resources hospitals already have

WHY THIS CAN SCALE

- Same core platform can support different hospital workflows
- Hospital-specific configuration creates deeper integration
- Additional modules can be added over time
- One successful ED deployment can become an entry point into wider hospital operations

“We are not selling another dashboard. We are building a decision-support layer around the hospital's most time-sensitive workflow.”

What Exactly Are We Selling?

PatientTriage.ai

INTELLIGENT TRIAGE

- AI-assisted initial assessment
- Dynamic P1–P5 prioritization
- Risk and confidence indication
- Identification of cases requiring human review

DYNAMIC ED OPERATIONS

- Continuously updated patient queue
- Deterioration-based reprioritization
- Waiting-time awareness
- Real-time operational visibility

RESOURCE ORCHESTRATION

- Doctor availability
- Treatment rooms
- Resuscitation bays
- Observation beds
- Diagnostic workflow

HUMAN-IN-THE-LOOP

- Clinician remains the final decision-maker
- AI recommendations can be overridden
- Override requires justification
- Decisions can be retained for auditability

“AI recommends. Clinicians decide. The platform connects that decision to the resources required to act on it.”

A Large Market with a Focused Entry Point

PatientTriage.ai

TAM

TOTAL ADDRESSABLE MARKET

- Hospitals with emergency-care operations
- Potential platform subscription
- Implementation and integration
- Future analytics and operational modules

SAM

- Have digital infrastructure
- Can integrate with the platform
- Operate within our initial geographic / commercial focus

SOM

- High-volume hospitals
- Digitally mature institutions
- Innovation-oriented hospital groups
- Hospitals willing to run controlled pilots

EXPANSION POTENTIAL

- Emergency Department
- Observation & Patient Flow
- Hospital Operations
- Multi-Hospital Intelligence

“Our entry point is narrow enough to be achievable, but the underlying problem is much larger than triage.”

Business Model & Go-To-Market

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HOW WE MAKE MONEY

- SaaS + Implementation
- Initial implementation fee
- Annual platform subscription
- Integration and configuration
- Optional premium modules
- Enterprise pricing for hospital groups

START WITH A PILOT

- Low-friction entry
- One Emergency Department
- Limited deployment period
- Defined operational objectives
- Measure before-and-after outcomes
- Use results to establish business value

CONVERT & EXPAND

- Successful pilot → annual contract
- Full ED deployment
- Additional workflows
- More users
- Additional hospital departments
- Expansion across hospital locations

WHO BUYS

- Primary stakeholders
- Hospital Operations
- Emergency Department leadership, Digital transformation / IT teams
- Internal champion
- ED leadership + hospital operations

"Prove value in one department → convert the hospital → expand across the hospital network."

Competitive Advantage & Moat

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WORKFLOW INTEGRATION

- Embedded into real ED workflows
- Connects triage, diagnostics and resources
- Becomes part of daily decision-making
- Deeper integration increases switching friction

HOSPITAL-SPECIFIC INTELLIGENCE

- Different staffing patterns
- Different resource constraints
- Different escalation processes

CONTINUOUS FEEDBACK

- Improved System
- Human overrides provide valuable feedback
- Operational outcomes can inform future improvements

COMPOUNDING ADVANTAGE

- More deployments
- More workflow understanding
- Better operational intelligence
- More value
- Stronger adoption
- More deployments

“Our moat is not simply the AI model. It is the combination of workflow integration, hospital-specific configuration, operational intelligence and continuous feedback.”

Thank you

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